

Services for Inclusion in Future Performance Years

Future years of the model would plan to expand prior authorization to additional clinical service areas, and new services would be added through an ICIP amendment. This could include services where prior authorization would have to be done on a more urgent or emergent basis, services that would require ‘bundling’ with other services, or services with more rapidly evolving clinical criteria, such as:

- Air ambulance transport (given the lack of non-emergent ambulance transport codes, untimely prior authorization of this service could result in a delay of medically necessary, emergent care)
- Advanced imaging (MRI, PET, and SPECT)
- Oncology care including the use of specific therapies, e.g., CAR T-cell therapy, radiation oncology
- Molecular and genetic lab testing
- Cardiac catheterization
- Electrophysiology procedures, e.g., pacemakers or ICD implantation

Lastly, future exploration could include items and services that do not have publicly available clinical coverage criteria in place, e.g., select high-cost Part B medications, which may require notice-and-comment rulemaking. Areas beyond physician services and ambulance transport could also be considered. Adding these services in the future would also allow the model to account for any new concurrent work happening in these areas across the agency, e.g., development of potential new CMMI model on Part B drugs and new CPI work targeting hospice Notice of Election, as described in 42 C.F.R. § 418.24(e), and other enrollment requirements.

6B. Payment Design

Payments to model participants would be based on the demonstrated reductions in spending for medically unnecessary or non-covered items or services, as defined by the Original Medicare local or national coverage policies and guidance. For each of the selected items or services, a participant would receive a percentage of the reduction in expenditures, or savings, that can be directly attributed to the prior authorization review process in their applicable region. These savings would be calculated from requests that did not result in a paid claim (i.e., non-affirmations not followed by an affirmed resubmission or a successfully appealed claim denial), multiplied by the average claim-level payments for historical regional claims submitted with the applicable item(s)/service(s) during the prior 12 months (adjusted for performance year pricing). The use of total claim payments in the savings calculation (as opposed to claim-line payments for the eligible CPT®/HCPCS codes) is intended to accurately depict the total revenue impact, including secondary or add-on services that are regularly billed and reimbursed along with the primary procedure or service codes. We are assessing several approaches to calculating average pricing, including historical prices, prices in the comparison group, or a blend of historical and comparison group prices.

Initially, model participants would receive model payments for non-affirmations that were not subsequently resubmitted and affirmed within 120 calendar days. A model participant will only be paid for one non-affirmation involving the same beneficiary, service/item, and provider/supplier within a period of 120 calendar days. For instance, if a provider/supplier resubmitted a request twice within 120 days of the original non-affirmation and neither resubmission was affirmed, the model participant would receive a single payment for the series of non-affirmations, not three separate payments. Payments for non-

These point allocations will remain in effect for subsequent performance periods unless CMS provides advance written notice of changes to the point allocation.

Aggregate Quality Score and Quality Multiplier

When calculating WISeR Payments, CMS applies a quality multiplier based on the WISeR Participant's AQS. CMS calculates the AQS as the percentage of points earned on quality measures among all points available on quality measures for that performance period. The AQS ranges corresponding to each value of the quality multiplier are provided below in Table 3.

Table 3. Aggregate Quality Score and Quality Multiplier

AQS Range	Quality Multiplier
85%-100%	100%
60%-84%	95%
<60%	90%

In the event of any changes to the quality multiplier or its corresponding AQS values, CMS will provide advance written notice before the performance period in which these changes take effect.

WISeR Quarterly Audit

CMS conducts quarterly audits of participant records in order to ensure that prior authorization determinations and pre-payment review determinations are consistent with Original Medicare coverage criteria for the relevant item or service. WISeR-2 (Accuracy of Determinations) is calculated from the results of this audit.

CMS audits every Select Item or Service at least once per calendar year and audits 25% of included services each quarter. To ensure that an equal number of Select Items or Services is audited per quarter within a given calendar year, services may potentially be re-audited within the same year. For the first three quarters of a year, items or services are randomly selected from the list of Select Items or Services that have not yet been audited in that year; in the fourth quarter, CMS audits the remaining services and (if necessary) randomly selects one or more services for re-auditing. The Select Items or Services to be audited for a given quarter are not communicated in advance to WISeR Participants.

For each performance period, WISeR Participants submit certain required data elements to facilitate reconciliation and quality scoring. These data document each prior authorization determination and each pre-payment review determination issued by the participant for the performance period. CMS uses these files to randomly select records for auditing among all determinations that a WISeR Participant issued for the relevant Select Items or Services.

Sampling is stratified by the Select Item or Service, the review method (prior authorization or pre-payment review), and the outcome (affirmation or non-affirmation for prior authorization; approval or denial for pre-payment review). Altogether, each participant can expect about 120 records to be selected for each Select Item or Service being audited in a given quarter. After selecting records for auditing, CMS will request all relevant documentation for these selected determinations from the WISeR Participant. (WISeR Participants are required to maintain



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Ms. Kate Blackwell
Division Director
Center for Medicare and Medicaid Innovation
Centers for Medicare and Medicaid Services
United States Department of Health and Human Services
7500 Security Blvd
Baltimore, MD 21244

Subject: Innovaccer Approach to Implementation and Go-Live on January 5, 2025

Dear Ms. Blackwell,

Innovaccer appreciates the time and discussion on Monday, December 8, 2025 to review our progress to date and the barriers to a successful January 5, 2026 go live. Below please find a summary of the discussion points, including Innovaccer's current go live status, the barriers and challenges hindering full functionality and our proposed phased capability rollout.

Innovaccer current Go Live status:

Innovaccer will meet the January 5, 2026 go live date. However, we will not go live with full functionality due to the constraints outlined below. When we initially go live, Innovaccer intends to **auto-affirm all prior authorization requests** while we continue to develop and deploy functionality.

Barriers to having a go live with full functionality:

As previously communicated, Innovaccer identified a narrow window in which MVP could be achieved by January 5, 2026. However, several critical assumptions around core integrations, requirements and testing did not materialize and/or materially changed throughout implementation. For instance:

1. **Evolving requirements changes** to critical workflow processes, such as ongoing updates to the esMD requirements document.
 - a. **Impact:** Necessitates **considerable rework** to development work that Innovaccer had previously tested.
2. **New core integration expectations** not previously identified during the planning phase, such as the HETS integration for beneficiary eligibility verification and PECOS for provider enrollment status.
 - a. **Impact:** This change required facilitating agreements and procuring DigiCert, along with **additional development and testing** not included in the initial schedule.
3. **Unclear governance processes**, version control and notification protocols, such as esMD system changes made prior to distributing documentation for those changes while testing is in progress.



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- a. **Impact:** The lack of governance processes creates unnecessary resource strain to troubleshoot and often requires rework, diverting critical resources from core delivery.
- 4. **Lack of end-to-end testing** and time for providers to prepare systems and processes ahead of go live. Providers and suppliers have internal workflows, connectivity, and documentation processes impacted by the WISeR model and the compressed timeline does not allow for end-to-testing with the provider community.
 - a. **Impact:** Inadequate provider preparation increases the potential for provider abrasion and disruption to beneficiary services.

Phased Capability Rollout

Innovaccer understands that the current go live date remains unchanged. With that in mind, we will proceed with a **Phased Capability Rollout**. Our approach means we will proceed with development to implement **auto-affirmation** capabilities for all WISeR Prior Authorization Requests and Prepay Claims by January 5, 2026. This approach ensures providers will still be able to submit and get a response for either a prior authorization request or prepay claim while we proceed to enable full capabilities in subsequent sprints. We expect to move out of our phased capability approach **within 45-60 days of going live**. Once the production solution is live, all new requests will be subjected to appropriate clinical review consistent with WISeR requirements and applicable coverage policies.

Given CMS's decision not to delay the model start date, auto-affirming is the only path available that avoids creating a backlog of unprocessed prior authorizations and claims while we finalize, validate, and deploy the full rules-based solution. This interim strategy is intended to last only as long as is strictly necessary to complete testing and ensure that the deployed workflows, clinical rules, and documentation handling are accurate and reliable.

This approach will, on a positive note, allow us to receive real clinical documentation so that we can appropriately test our solution prior to release, reducing potential abrasion that may otherwise be experienced without proper testing.

We acknowledge that auto-affirming comes with some concerns such as potential increase volume in appeals once transactions are processed against clinical criteria, potential provider complaints when there are authorizations approved during the auto-approve period and similar requests/claims are not affirmed at a later point, and potential impacts to Gold Carding.

- **Increased Appeal Volume:** There is a risk of increased appeals volume if early-period approvals differ from later determinations once full review is in place. We will work with CGS and CMS to flag the impacted time window and maintain clear audit trails so that any appeals arising from this period can be identified and handled with full awareness of the interim processing methodology.
- **Provider Abrasion/Confusion:** Providers may question why similar documentation may be handled differently before and after Go-Live of full medical review. Innovaccer will support consistent provider messaging (in coordination with CMS and CGS) that messages that ongoing refinement of the solution will occur throughout the performance period and providers should expect to see adjustments in returned decisions, particularly in the first 3-6 months.

MAC other than Noridian. This was forwarded to the CORs and a response that Virtix's question was forwarded on to CMS was sent back to the model participant. This topic was discussed at the 3/18/26 CMMI weekly meeting with the MACs.

- esMD went down sometime Thursday evening and was not fully up and running until close to 10pm EST on Friday. This delayed receiving responses from the model participants until late Friday and UTNs were not created until Monday due to the lateness of the receipts.
- Amy requested documentation of the requests for the backlog completion in reference to Cohere and Genzeon. This is being worked on based on the requests sent in the month of March. Once completed this will be forwarded.
- Genzeon Readiness:
 - As of COB 3/30/26, there are 123 Prior Auth requests (PARs) pending that are aged 3 days or older. The oldest case is 83 days old. 14 are still awaiting a model participant response in which the PAR was received by Novitas. 44 PARs are awaiting a decision letter after the UTN was sent to Genzeon. 65 are awaiting correction either for creation of a UTN or dismissal. They are making progress, but each week there are some errors or issues preventing them from getting completely caught up. Last we were told they would be completed by 3/31/26.
 - As of 3/30/26, there are 82 pre-pay review claims that are aged 46-50 days. Genzeon continues to work on the older claim sand were sent the list of the day 49 and 50 claims today.
- Cohere Readiness:
 - As of COB 3/30/26, there are 51 PARs pending that are aged 3 days or older with the oldest being 32 days old. 18 are awaiting a model participant response for a PAR that was received by Novitas. 16 are awaiting a decision letter for UTNs sent to Cohere, and 15 are awaiting correction for either UTN creation or dismissal by Cohere.
 - As of 3/30/26, there are 33 pre-pay review claims that are aged 46-49 days.
- Humata Readiness:
 - As of COB 3/30/26, there are four PARs pending that are aged 3 days or older. All are awaiting a decision letter for UTNs sent to Humata.
 - As of 3/30/26, there are 20 pre-pay review claims that are aged 46-56 days. For the day 56 claim, Humata identified that they sent the ADR a month after receiving the claim and will be reaching out to the provider to see if they can get a response in ASAP.

Ongoing Activities

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WISeR Weekly Report

Table 1 – Timeliness of Direct PA's Sent to WMPs Month to Date – March 2026

Days Old	Virtix (WA)	Zyter (AZ)	Totals	% Of Workload
1-5 days	1,317	4,393	5,710	82%
6-10 days	850	247	1,097	16%
>10 days	87	96	183	2%
Totals	2,254	4,736	6,990	100%

Table 2 – Year to Date Inventory Summary 2026 – Prior Authorization

Category	Virtix (WA)	Zyter (AZ)	Totals
UTNs Sent	5,670	14,163	19,833
Decisions Received	6,096	14,301	20,397
Pending Decisions	31	1,522	1,553

Table 3 – Summary of Year-to-Date Decisions Received – Prior Authorization

Category	Virtix (WA)	Zyter (AZ)	Totals
Affirm	2,863	11,590	14,453
Non Affirm	3,233	2,711	5,944
Total	6,096	14,301	20,397

Trending and Analysis

Year-to-date inventory trends show close alignment between UTNs Sent (19,833) and Decisions Received (20,397), indicating strong overall throughput and a consistent ability to convert intake into determinations.

- Virtix (WA) continues to show decisions exceeding UTNs sent (6,096 decisions vs. 5,670 UTNs), reinforcing a trend of effective processing and limited backlog accumulation.

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WISeR Weekly Report

conversations with the MACs and CMMI mentioned above. The most common is a missing diagnosis. Noridian Mitigation efforts underway include:

- Updating NMP to make the diagnosis field a required entry.
- Adding a pre-implementation message in NMP encouraging providers to complete the diagnosis field.-implementation message in NMP encouraging providers to complete the diagnosis field.

MEDIUM RISK: Noridian is currently utilizing a manual workflow to generate information required to transfer claims requiring ADR to the WMP. Approximately 470 claims have been received to date. Noridian initially anticipated being able to automate this process, however through the process of solutioning it was determined that we may not be able to. We are working through the workload and sending claims to the WMP. Noridian is prioritizing the manual workflow to ensure claims are submitted to WMPs promptly for processing.

Noridian continues to closely monitor all low and medium risk items and remains committed to timely mitigation actions to support stable WISeR operations.

Inventory Updates

Noridian continues to successfully submit prior authorization requests with UTNs to the WMPs and receive both direct PAs and corresponding decisions. Inventory levels are closely monitored to maintain visibility into WMP decision activity and support timely processing. The timeliness has increasingly improved.

Table 3– Timeliness of Direct PA's Sent to WMPs in January 2026

Days Old	Virtix (WA)	Zyter (AZ)	Totals	% Of Workload
1-5 days	700	188	888	70%
6-10 days	218	44	262	20%
11-21 days	112	14	126	10%
Totals	1,030	246	1,276	